

Chronic Atrial Fibrillation Management — Clinical One-Pager

Stroke prevention · rate control · rhythm control. Framing per 2023 ACC/AHA/ACCP/HRS AF guideline. Adapted from TeachIM.org (CC BY-NC 4.0). Educational use only.

1 · Stroke risk → anticoagulate

CHA₂DS₂-VASc: CHF 1 · HTN 1 · Age 65-74 =1 / ≥75 =2 · DM 1 · Stroke/TIA/embolism 2 · Vascular dz 1 · Sex (female) 1.

Anticoagulate if annual stroke risk ≥2%/yr (Class 1) ≈ ≥2 men / ≥3 women. Score 1(m)/2(f) = reasonable (2a). Use score to estimate risk, not as rigid cutoff. **ASA not** for stroke prevention. Any generalist can start it.

2 · Pick the anticoagulant

Situation	Choice
Most patients	DOAC (Class 1, >warfarin)
Mechanical valve / mod-severe MS ("valvular")	Warfarin only
ESRD / CrCl <15	Apixaban or warfarin
BMI >40 (to ~50)	Apixaban or rivaroxaban
Stable CAD >1y from MI/PCI	OAC alone (no ASA)
Can't take OAC long-term	LAAO/WATCHMAN (2a)

AS, MR, bioprosthetic = **non-valvular** → DOAC OK.
Apixaban reduce to 2.5 mg BID if ≥2 of: age ≥80, wt ≤60 kg, Cr ≥1.5.

Don't withhold OAC for...

High HAS-BLED or frequent falls (alone). High bleeding-risk pts often gain **more** absolute benefit; ~295 falls/yr needed to outweigh. **HAS-BLED:** HTN(uncontrolled) · Abn renal/liver · Stroke · Bleeding hx · Labile INR · Elderly>65 · Drugs/EtOH.

Cardioversion rule

AF >48 h / unknown → **3 wk OAC or TEE before**, then **4 wk OAC after regardless of CHA₂DS₂-VASc** (atrial stunning).

3 · Rate control

Goal HR <110 bpm most (RACE II); lower if symptomatic / declining LV / tachy-cardiomyopathy.

- **Beta-blocker** (metoprolol succinate, carvedilol, bisoprolol in HFrEF)
- **Diltiazem** (non-DHP CCB) — **contraindicated in HFrEF**
- **Digoxin** adjunct (HFrEF; renally cleared, watch tox)
- Refractory → AV-node ablation + pacemaker

4 · Rhythm control

Favored by: symptoms attributable to AF (strongest), younger age, few comorbidities, recent onset, can't rate control, tachy-cardiomyopathy, LV dysfunction.

2023: catheter ablation **Class 1 first-line** in selected paroxysmal AF & in AF+HFrEF.

AAD tox: amiodarone → lung/liver/thyroid (PFT/LFT/TFT); sotalol & dofetilide → QT/Torsades (inpatient start, renal dosing); flecainide → avoid in structural/CAD.

2023 staging & risk-factor pillar

Stage 1 at-risk → 2 pre-AF → 3 AF (3A parox / 3B persistent / 3C long-standing / 3D post-ablation) → 4 permanent. **Lifestyle:** wt loss BMI <27, ↓/stop alcohol, BP & lipids, OSA screen, tobacco cessation.

A&P dotphrase

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#AF (chronic) *[parox/persist/perm; stage *]
- CHA2DS2-VASc ** → OAC if ≥2%/yr (≥2 M/≥3 F).
  periCV x4wk always. no ASA.
- HAS-BLED * — don't withhold for score/falls
- DOAC 1st * [apix/riva/dabi/edox]; apix if
  CrCl<15/ESRD or BMI>40; warfarin only mech valve/
  mod-sev MS
- CV: AF>48h → 3wk OAC or TEE, then 4wk after
- rate <110 *: BB or diltiazem (NOT in HFrEF);
  digoxin adjunct
- rhythm if sx*/young/recent/tachy-CM/LV dysfn →
  cards; ablation Class1 1st-line select parox &
  AF+HFrEF
- AAD *[amio(PFT/LFT/TFT)/sotalol·dofet(QT,inpt)/
  flecainide(no struct)]
- LAAO 2a if OAC contraindic; RF mod: wt/EtOH/BP/
  OSA
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